Inspire London College

Unit – An Introduction to Social Health and Care

Study Manual

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Introduction to Health and Social Care

Healthcare:

All the steps, such as diagnosing, treating, and preventing mental and physical illness, come under the umbrella of healthcare. These are the services provided by the people to the people to promote wellbeing. In the UK, the National Health Services (NHS) is the institution that provides the basic healthcare services free of cost for the adult citizens; emergency service, first aid, prescription, seasonal illness, etc. Moreover, services like dental care and eye tests are not included in this.

Social care:

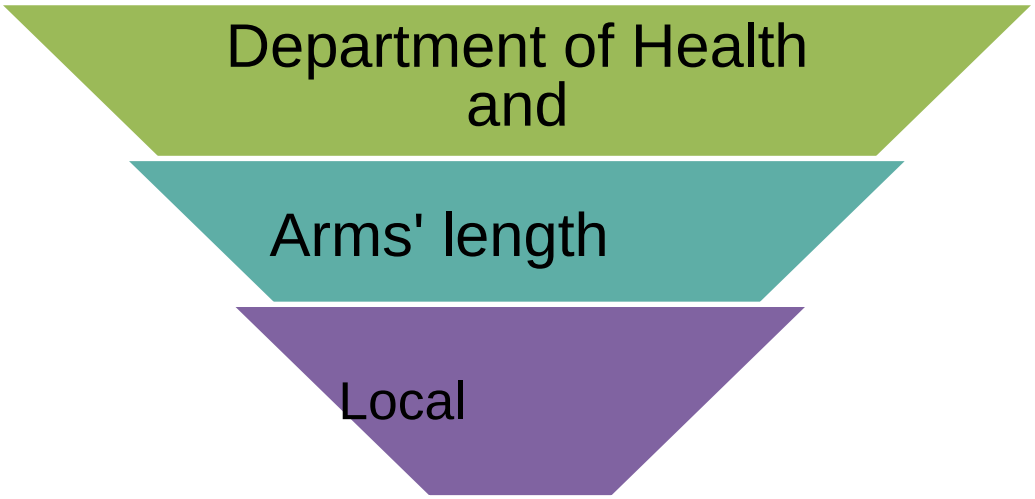
The services that are composed of more community care services than healthcare services are termed social care. These services include day-care services, community centres, aid at home, mental health, etc. For example, a person who is mentally unsound needs social care, which could be provided by a friend or family member. In the UK, local authorities are responsible for providing citizens with social care services.

Continuing Healthcare:

The free of cost service provided at home for your physical and mental health is referred to as continuing healthcare. A person is entitled to it only if he/she has either physical and mental illness or both. He/she is not entitled to receive continuing healthcare for mental illness alone. Oftentimes, the service is provided by the local commissioned service provider such as nurses, social carer, etc.

Hierarchy of Health and Social Care in the UK:

Health and social care in the UK are divided into three bodies; the department of health and social care is the main governing body, then comes to the arms-length bodies, and then local authorities that are responsible for providing health and social care services to people. Let us look briefly at the hierarchy of these bodies.



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Department of Health and Social Care:

Ministry of Health Act 1919 initiated the first government body that was the Ministry of Health. It was responsible for the health and wellbeing of the nation. Till 1945, the Ministry of Health coordinated with local bodies to provide emergency and wartime services. The consequent development of the Health department resulted in the formation of the National Health Services (NHS) in 1948.

Later on, the government created the department of health and social security (DHSS) from the dissolution of the Ministry of health. After twenty years, two government departments were formed from the department of health and social security, namely, the department of health and the department of social security. These departments were concerned with their own particular area of health and social care. Later on, these were merged again and renamed the department of health and social care.

The department of health and social care (DHSC) is liable for governing policies on health and adult social care. Three members of parliament and three ministers of state leads the department of health and social care. It deals with health and social care matters in England and regions that are not yet devolved to the Scottish and Welsh governments.

The department of health and social is the main governing body for the matters of health and social care of adults. It creates policies for the betterment of the people concerning their physical and mental health. The department of health and social care executes its policies via arms-length bodies (ALB). The arms- length bodies are both executive non-departmental public bodies and executive agencies.

Is Adult Social Care Free for Everyone?

Local authorities are responsible for providing adult social care, and it is not free for everyone. Funding for adult social is based on the social condition of an individual. Local authorities or the health and social care provider assess the patient and determine whether he/she is eligible for social care funding or not.

Regulation

The Care Act 2014 is the result of modifications in the acts since 1948. It started from the National Assistance Act 1948, proceeded by Chronically Sick and Disabled Person Act 1970, etc. The Care Act 2014 is functional from 2015. The development of this act has brought it to the level of maturity that enabled people to manage their care and support efficiently. Before the Care Act 2014, the people of Britain were facing hardship to understand the hierarchy of health and social care services.

This act made things simplified for people, so now they can better understand the mechanism of health and social care services. Now, people are aware that who is entitled to social care provision; under certain circumstances. People understand the role of executive bodies, local authorities, and the providers of health and social care services to people. Let us discuss the act briefly so that you can have an idea of the flow of duties and rights of different stakeholders;

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government, arms-length bodies, local authorities, and people.

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The Care Act 2014 is divided into parts; Care and Support, Care Standards, Health, Health, and Social Care, and General. Let us discuss it in parts, thus, to have a better understanding.

Part 1: Care and Support

The first part of the Care Act 2014 targets the Care and Support for the people. It consists of the general responsibilities of local authorities to ensure individual wellbeing, meeting the current health and social care needs of the people. This part provides information and advice regarding the development of the healthcare workforce and equipping the nation with facilities to meet the future need for health and social care. It further addresses the criteria for the assessment of an adult's needs for social care and support. It also informs about the carer's needs for support; the carer maybe a friend or a family member.

Part 2: Care Standards

The second part of the Care Act 2014 deals with the quality of health and care services. It targets NHS foundation trusts, the appointment of administrators, warning notices, etc. This part contains literature about care quality commission regarding rights of appeal. The Care Standards part of the Care Act 2014 emphasises the independence of the Care Quality Commission. Moreover, this part of the act provides information about performance rating, review, and assessment of performance.

Part 3: Health

The third part of the Care Act 2014 provides information about Health Education; primarily, the health education England and the local education and training board. It targets the improvement of education, its provision, and need. This part of the act emphasises quality health education for the healthcare workforce. It further describes the establishment of the Health Research Authority, its general function, and practices. Moreover, it sheds light on the Health Research Authority's policy regarding the research ethics committee.

Part 4: General

The fifth part of the Care Act 2014 provides information about the general practices of health and social care. It sheds light on general interpretation, regulations, and orders. Moreover, it informs about the adult boards, the procedure of discharging a patient from the hospital, and direct payments to the patients under the Mental Health Act 1983.

Child's Care and Support:

According to the Care Act 2014, local authorities are responsible for providing care and support to children. Local authorities will make assessments regarding needs for support and decide whether he/she needs support or not. The assessment should include the child's future needs for support. Local authorities must involve the parents of the child or carer while carrying out a needs assessment.

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Payments:

Personal budget and direct payments are in favour of adults seeking care and support. Under the Care Act 2014, the personal budget will be allocated to an adult, which will cover the needs for care and support. Moreover, direct payment means that an adult will have full control over his/her care and support. He/she can avail services of his/her own choice.

Care plan:

The care plan of an adult means the range of available care and support services. Care planning and review have enabled an adult to better avail of care and support services. Time to time review of the care plan will ensure that plan is up to date with an individual's situation and need(s) of requirement.

When is an adult eligible for care and support?

According to the Care Act 2014, local authorities must consider these three conditions in an adult seeking health and support services. These conditions include physical or mental illness, performing a task resulting in an undesired situation, and overall affected wellbeing.

Physical or Mental illness:

An adult with physical or mental illness is eligible for care and support. This may include learning disability, brain injury, sensory, mental, and physical illness. Local authorities must assess the support seeker for this condition.

The resulting need for support:

If an adult is in a situation that performing a task can result in an unwanted circumstance, he/she is eligible for support. An example includes; if an adult can drive, but doing so will leave him/her in severe pain, he/she is eligible.

Affected wellbeing:

Promotion of wellbeing is the motive of the Care Act 2014, and if the needs of an adult are contributing towards ill-being, he/she is eligible for care and support.

When is a Carer eligible for support?

Under the Care Act 2014, the carer is also eligible for support under specific conditions. These conditions include eligibility of an adult to whom the carer is caring, their wellbeing is at risk because of caring, and the effect of duties on the carer's wellbeing.

Patient (People) Rights under the Care Act 2014:

The health and social care providers are responsible for the following duties regarding the patient's rights
to comply with the regulation.

- The patient better know his/her state or condition

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- The health and social care provider is responsible for considering the patient's beliefs, values, and
  thoughts
- The health and social care provider must focus on the patient's wellbeing and try to reduce the
  need for future treatment
- The health and social care provider cannot make any decision for the patient without his/her acknowledgement
- The health and social care provider must equip the patient's personal carer with the ability to treat him/her in a better manner
- The health and social care provider must not breach your rights except under particular circumstances

Poor Practice in Healthcare

There remains room for quality in everything; goods, services, and sectors. There is a possibility of poor practice in the care sector; thus, the Care Quality Commission (CQC) is working to maintain a high level of quality standards in the care sector. This may include training and educating healthcare staff, raising awareness programs for the public, etc. The key thing regarding poor practice in the healthcare sector is whistleblowing. Whistleblowing is an act of raising concern regarding something illegal within the organisation.

Poor Practice:

The failure of healthcare staff to provide satisfactory healthcare service is called poor practice. It happens because of healthcare staff's negligence towards organisational policies and procedures and results in harm and abuse of an adult.

Good Practice:

Good practices in the health and social care sphere are providing the best possible care facilities to individuals, such as treating them gently, understanding their emotions, etc.

Duty of Care:

A duty of care in the health and social care sphere is the minimum expected services that should be provided by healthcare professionals (doctors, nurses). This duty includes that the healthcare provider should limit his/her act within competencies and do not involve in things that they can't handle.

What are the types of abuses?

There are different types of abuses that can happen as poor practice in the health and social care domain. For example, physical abuse, emotional abuse, sexual abuse, financial abuse, and neglect.

How to report?

If you find any poor practice at the workplace that you believe is against organisational policy and procedures, report it to your up line. You can report to the supervisor or line manager.

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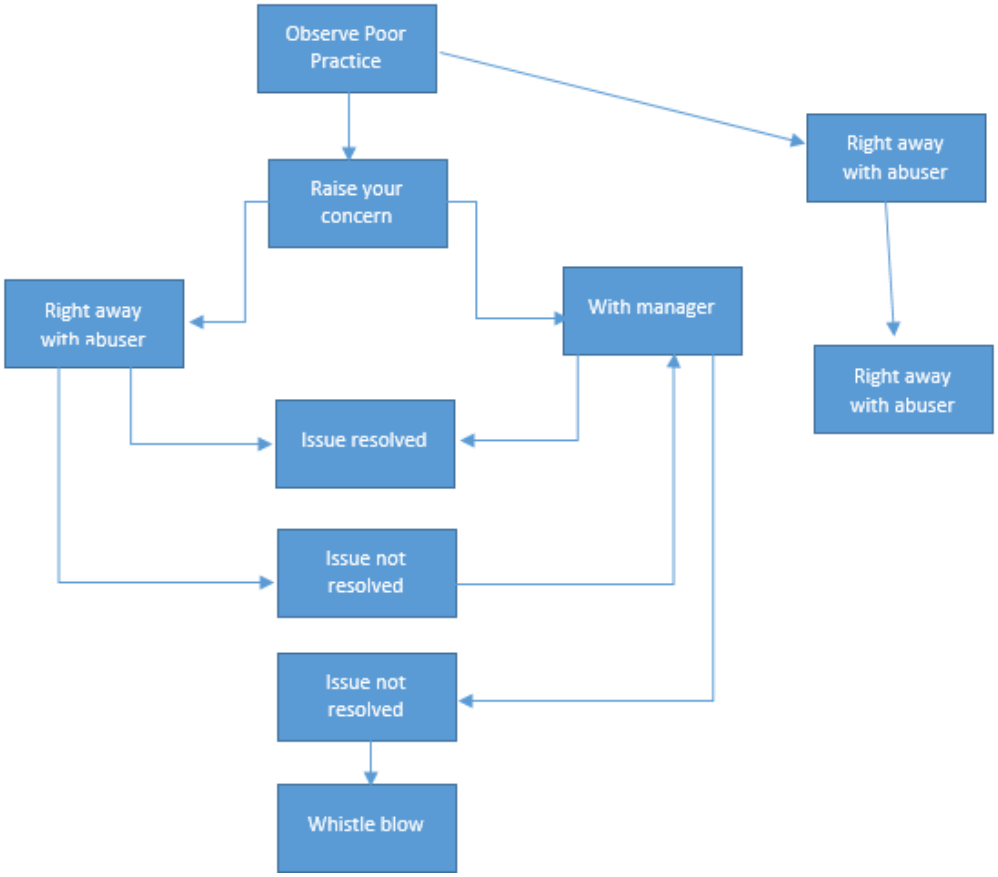
You can consider these guidelines while reporting poor practices.

Assessment of the situation:

You should not act immediately in any situations that seem poor; rather, you should carefully assess the situation that what has happened or what is happening. After observing carefully, take the next step.

Explore option(s):

Now, you will explore options according to the circumstances and your potential. For example, you see a nurse misbehaving with a patient; if you have authority to object to this, you can ask him/her to treat the patients gently.



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Arms-length bodies (ALB):

Arms-length bodies provide a helping hand in governing health and social care policies. There are different arms-length bodies that have a significant purpose, ranging from licencing and regulating medicines to the training health workforce. The department of health and social care has two types of arms-length bodies; first, executive agencies that are responsible for carrying out ministerial orders and policies, and second, executive non-departmental public bodies that are sponsored by the department to carry out assigned work at the arms-length from ministers. Let us look at executive agencies and executive non-departmental public bodies.

Executive agencies:

There are two executive agencies of the department of health and social care that assist the department in carrying out orders and act according to policy. They are led by the ministers of the department of health and social care. The two executive agencies are;

Medicines and Healthcare Products Regulatory Agency:

An executive agency which is responsible for the regulation of medicines and products under the supervision of the department of health and social care is known as the Medicines and Healthcare products Regulatory Agency (MHRA). It is responsible for ensuring that medical products are safe for use.

The Medicines and Healthcare products regulatory agency was found in 2003 by the venture of Medicine Control agency and the Medical devices agency. It has a workforce of more than 12,000 peoples. The main duties of the medicine and healthcare products regulatory agency are; assessment of medical products, likening and regulation of medicines and other products.

Public Health England (PHE):

The Second executive agency of the department of health and social care is Public health England (PHE), which is responsible for raising awareness of protection and infection control among the masses. Public health in England was founded in 2013. According to the care act 2014, the role of the health protection agency and the national treatment agency is shifted to the public health England. The public health England is autonomous in its operation.

Executive non-departmental public bodies (ENPB):

The department of health and social care has thirteen executive non-departmental bodies dedicated to different regions. These bodies are not a direct part of the government department;

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rather, they serve the government department. They carry out the departmental functions by further cooperating with public bodies. Executive non-departmental public bodies are sponsored by the government departments but work independently. Let us briefly discuss some of these public bodies responsible for health and social care in the UK.

Human Tissue Authority:

The Human Tissue Authority is responsible for regulating the usage of human tissue in research in the UK. This body particularly targets researches and therapeutic treatments.

Human Fertilisation and Embryology Authority:

The Human Fertilisation and Embryology Authority are responsible for the regulation and inspection of artificial insemination and the storage of human eggs. This body also deals with the regulation of human sperm or embryos. The Human Fertilisation and Embryology Authority particularly targets the research on human embryo and fertilisation. It is functional in the UK.

NHS Blood and Transplant:

The NHS Blood and Transplant control the blood supply, management of human organs and tissues. Its main concerns are the supply, storage, donation, and transportation of the blood, organs, stem cells, and tissues of humans. This non-departmental body is working in England and Wales.

National Institute of Health and Care Excellence:

The National Institute of Health and care holds the responsibility for assessing healthcare procedures for cost-effectiveness. This body also provides guidance related to treatment procedures to optimise the process. It is functional in England and Wales.

NHS Business Services Authority:

The NHS Business Service Authority is the body that serves the National Health Services. It is responsible for providing business support services to NHS. The NHS Business Services Authority oversees the Services for NHS such as management of employees' electronic record, helping NHS with health costs, management of NHS jobs, etc.

Care Quality Commission:

The Care Quality Commission is responsible for registration, monitoring, and inspection of the health and social care public bodies operating across England. The sole purpose of the Care Quality Commission is to ensure that these health and social care providers are meeting the quality criterion. This body controls the registration process of service providers, such as individuals and organisations.

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NHS Digital:

The NHS Digital is the new name of Health and Social Care Information Care and is responsible for the management of information regarding the health and care sector. It collects and analyses the information needed for health and social care departments to carry their functions. The NHS Digital issues the information standards for the partner bodies to comply.

Health Education England:

Health Education England is dedicated to the development of a quality healthcare workforce. This body oversees the allocation of resources needed to meet the current and future needs in the health and social care sector. It is responsible for ensuring the wellbeing of people and for the promotion of health and social care services by utilising digital means.

NHS England:

The NHS England is responsible for monitoring and controlling the NHS extensions such as NHS Digital, NHS improvement, etc. As its name suggests, it is operational in England only. NHS England holds the responsibility of controlling clinical commissioning groups that are the organisations created for the arrangement of NHS services' delivery.

NHS Improvement:

NHS Improvement is responsible for controlling NHS trusts. It monitors the independent providers also who provide NHS-funded care. The sole responsibility of NHS Improvement is to ensure financially sustainable healthcare to the patients, taking into account the high quality of standards. The NHS is also responsible for managing the National Reporting and Learning System. The NHS Trust Development Authority works in collaboration with NHS Improvement; however, they have independent budgets.

NHS Resolution:

The NHS Resolution is responsible for the management of negligence claims such as negligence of a healthcare provider. This helps to improve the quality of healthcare delivery, learning lessons from past negligence claims and poor healthcare practices.

NHS Counter Fraud Authority:

The NHS Counter Fraud Authority is responsible for dealing with fraudulent activities concerned with corruption. This body fights against fraud and corruption, ensuring high-quality standards. The NHS Counter Fraud Authority is directly sponsored by the department of health and social care's anti-Fraud Unit.

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Local Authorities

Since April 2013, local authorities are providing health and social care services to deserving local citizens. These services include sexual health, reduction of drug usage, a shelter for the needy, etc. Moreover, the responsibility of local authorities in general includes:

- promotion of individual wellbeing
- minimising the future needs for care and support
- provision of information and advice
- promotion of diversity and quality while providing services
- cooperation in specific cases
- general cooperation with adults

Let us briefly discuss these responsibilities of local authorities.

Promotion of individual wellbeing:

The local authorities are responsible for promoting individual wellbeing. They must consider both; the physical as well as emotional wellbeing of a patient under treatment. Local authorities must ensure that patient is treated well and protected from negligence and abuse. Moreover, local authorities should take care of patient's daily care that may include support to individuals or carers. Local authorities must participate in education, work, and training regarding health and social care.

Delaying or Preventing future health and care needs:

Local authorities are responsible for taking measures to delay the health and social care needs and prevent them from occurring in the future. Reduction of carer's needs for support is also the responsibility of local authorities. This is attainable by providing education and training to equip the carer with the required skills of support, thus, minimising the future needs. Furthermore, local authorities must identify the available resources and services to ensure the availability of services at the least possible cost. Local authorities may charge for the additional or other services that are exempted from basic health and social care services. These additional services may include specific services provided under certain circumstances.

Provision of Information and Advice:

Local authorities are responsible for providing information and advice service, information to adults regarding health and social care, and information or advice on support to the carer. This information may include the working procedure of the authority, the mechanism of service provision, etc. The information regarding service by the provider and means to access or avail

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that service. Furthermore, local authorities must provide information related to different situations that need social care, such as addiction to drugs, emergencies, etc.

Promotion of diversity and quality while providing services:

Local authorities are responsible for promoting diversity in the provision of health and care services. They must ensure that quality standards are met while providing service. They must ensure that adults have the option to choose to form a variety of service providers. This will allow adults to choose the provider of their choice, thus, promoting the diversity and quality of in health and social care sector. Furthermore, local authorities must make arrangements for volunteer training and education programs, thus, providing a platform for those who want to contribute to social care services.

General cooperation with adults:

Local authorities are responsible for cooperating with adults and other relevant partners to ensure smooth delivery of health and social care services. Under this responsibility, means are required to facilitate the authority officer regarding the execution of the health and support functions; for the adult and the carer. This may include, promotion of the wellbeing of adults regarding the need for care and support. Moreover, general cooperation means the elevation of quality standards in the provision of health and care services. Local authorities should also take into account the risk of abuse and negligence of adults. They must learn from the lessons and take measures to minimise them in the future; cases of adult abuse and negligence.

The relevant partner of a local authority may include, NHS body in the authority's area, the minister of the crown governing social security, and a person who is the provider of probation service.

Cooperation in specific cases:

This part says that when local authorities request cooperation from relevant partners for the provision of health and social care services, they must cooperate. The partnered service provider may refuse the cooperation under specific cases, such as when doing so is in favour of the adult or the carer. The relevant partner is responsible for providing written refusal regarding the cooperation request.

Local authorities meeting the needs:

Local authorities may meet the needs of adults by providing accommodation service, care, and support, goods, information, or advocacy. Local authorities can provide the accommodation service either in a care home or on some other premises. They can provide counselling to meet

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the need for social care. Local authorities may provide information relating to services they provide or the mechanism to avail health and social care service.

Local authorities may provide the services with their selves or arrange a provider according to specific circumstances. They can pay directly for the service or arrange the provision of service in the care home or any other concerned premise.

How Local authorities assess the needs of an adult?

Local authorities may assess the need of an adult for the health and social care provision by following methods. They can assess him/her visually whether he/she needs health and social care support or not. If yes, then what is the condition of an adult; is it critical enough to be treated immediately, is it moderate so that to schedule it. Local authorities also assess the financial condition of the adult. While assessing, local authorities must involve the adult and his/her carer in the assessment.

Assessing carer's needs for support:

Local authorities may assess the carer for support needs. They may enquire whether the carer has needs for support or there is any possibility of future needs. If the carer does require support, local authorities will then identify what those needs are. They will assess whether the carer is willing to support, and will he/she be willing to provide future care or not. The assessment will include the wishes of the carer, thus, to know whether the carer can meet his/her wishes by the provision of care and support or not.

Health and Social Care Teams

A team is a group of people formed to achieve a goal. Teams can be temporary or indefinite. With individuals sharing responsibility, the group as a whole can take advantage of all of the collective talent, knowledge, and experience of each team member.

Interdisciplinary Team:

A group of healthcare professionals with dedicated responsibilities and significant expertise is called an interdisciplinary team. This team may consist of doctors, carers, patients, therapists, and other health and social care professionals. The team member works collaboratively towards their goal.

Multidisciplinary Team:

A group of healthcare professionals with a different set of expertise, who treats the patient according to their area of expertise is called a multidisciplinary team. The team may consist of social workers, psychiatrists, doctors, carers, and patients. The working procedure of the multidisciplinary team is a different form of the interdisciplinary team in the sense that the healthcare professionals crate their own set of goals for the patient, and this could result in conflict.

Benefits of Interdisciplinary teams:

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Inter-professional or Interdisciplinary teams are beneficial for achieving patient's health goal(s). In the inter-professional team, the professional works toward the same agreed-upon goal of patient's health. When professional with a different set of expertise join their head to create a tailored care plan for an individual, the probability of goal achievement increases.

Inter-professional teams are beneficial because:

- The team member works collaboratively
- They work toward the same goal
- They create a care plan according to their expertise
- They facilitate each other with their knowledge

Barriers to Inter-professional teams:

Inter-professional teams are beneficial for the provision of health and social care services; however, there is some common barrier to interprofessional healthcare teams. Those barriers include an organisational barrier, inter-team barriers, and personal barriers. Let us discuss them briefly.

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Organisational Barrier:

The organisational level barrier can include different aspects such as financial constraints, regulations, and organisations' vision and mission. The organisational barrier affects the performance of the inter-professional healthcare team. If the roles of professionals are not appreciated at the organisational level, it degrades the performance.

Team Level Barriers:

When there is a conflict between team members, it is hard to purse the health goals of patients. Team level barriers may contain team structure, lack of communication, language barrier, gender barrier, discrimination, influence, office politics, and ambiguous leadership.

Personal Barrier:

There is a personal situation that affects the performance of inter-professional healthcare teams. For example, lack of interest in teamwork, change reluctance, defensive attitude, lack of confidence, and disloyalty.

How to overcome inter-professional team barriers?

Measures to overcome inter-professional team barriers depends on the nature of the barrier. Organisational barriers need an organisational level measure to eradicate barriers, such as increasing the budget to eliminate financial constraints. Following are the general measures to overcome barriers:

- Development of trust across team members
- The flow of information within the team
- Improved communication
- Conflict resolution

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